National Cancer Control Programme (NCCP) 00278 · clinician draft

Dose-dense AC → paclitaxel every 2 weeks

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

AC · 14-day schedule

Active medicines: Doxorubicin + Cyclophosphamide

Emetogenic risk: High · AC

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1
Ondansetron	16 mg orally	Day 1 Individual dose / QT / hepatic review
Dexamethasone	12 mg orally	Day 1 No routine Days 2–4 dexamethasone in this National Cancer Control Programme (NCCP) AC antiemetic row
Olanzapine	5 mg orally	Days 1–4 Review sedation, falls, interactions and glucose
Aprepitant	80 mg orally daily	Days 2–3
Metoclopramide	10 mg orally up to three times daily	Subsequent days: duration and use to be confirmed Caution with olanzapine: additive extrapyramidal effects; adjust / avoid for individual contraindications

Additional phase-specific support

- granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) is part of each dose-dense AC cycle; prescribe product and timing separately.
- This AC antiemetic script is replaced at the taxane transition.
- At the AC-to-paclitaxel transition, stop using this AC antiemetic template and select the taxane phase. Review residual symptoms separately.
- Dose-dense treatment requires the prescribed G-CSF plan. NCCP requires G-CSF with all cycles and refers to local policy; specify the chosen product, dose and days in the individual prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00278 v7 · p6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-23 · SACTCheck v0.84.17 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Taxane / combination phase

Active medicines: Paclitaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- Fortnightly paclitaxel: dexamethasone 20 mg orally at 12 and 6 hours before treatment, OR the source intravenous (IV) option 30 minutes before; chlorphenamine 10 mg IV and famotidine 20 mg IV before treatment. The team selects the route / timing; do not substitute the weekly 8 mg first-dose schedule.
- Growth-factor support is required for the dose-dense taxane phase. Consultant-directed steroid reduction / omission must account for reactions and interactions; if aprepitant is added, the source discusses 12 mg treatment-day dexamethasone.

- Dose-dense treatment requires the prescribed granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) plan. National Cancer Control Programme (NCCP) requires G-CSF with all cycles and refers to local policy; specify the chosen product, dose and days in the individual prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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